

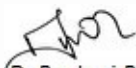
Patient Name : Mr.LAKSHMANAN
Age/Gender : 44 Y 10 M 2 D /M
UHID/MR No : CVAL.0000050587
Visit ID : DKUMOPV7164
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 21/Aug/2026 12:09PM
Received : 21/Aug/2026 01:37PM
Reported : 21/Aug/2026 03:01PM
Status : Final Report
Client Name : PUP 24X7_CREDIT
Center location : 600056,Chennai

DEPARTMENT OF HAEMATOLOGY
XPRT HEALTH ANNUAL HEALTH CHECK-UP

Test Name	Result	Unit	Bio. Ref. Interval	Method
HEMOGRAM , WHOLE BLOOD EDTA				
HAEMOGLOBIN	11.8	g/dL	13-17	Spectrophotometer
PCV	36.30	%	40-50	Electronic pulse & Calculation
RBC COUNT	4.4	Million/cu.mm	4.5-5.5	Electrical Impedance
MCV	82.4	fL	83-101	Calculated
MCH	26.9	pg	27-32	Calculated
MCHC	32.6	g/dL	31.5-34.5	Calculated
R.D.W	16.4	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	5,500	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	56.9	%	40-80	Flow cytometry
LYMPHOCYTES	37.2	%	20-40	Flow cytometry
EOSINOPHILS	1.4	%	1-6	Flow cytometry
MONOCYTES	4.4	%	2-10	Flow cytometry
BASOPHILS	0.1	%	0-2	Flow cytometry
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	3129.5	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	2046	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	77	Cells/cu.mm	20-500	Calculated
MONOCYTES	242	Cells/cu.mm	200-1000	Calculated
BASOPHILS	5.5	Cells/cu.mm	0-100	Calculated
Neutrophil lymphocyte ratio (NLR)	1.53		0.78- 3.53	Calculated
PLATELET COUNT	219000	cells/cu.mm	150000-410000	Electrical impedance
MPV	8.5	fL	8.1-13.9	Calculated
ESR	5	mm at 1 hour	0-15	Modified Westergren

Page 1 of 7


Dr.Rashmi R
M.B.B.S.,M.D(Pathology)
Consultant Pathologist

SIN No:HA11866018



Patient Name : Mr.LAKSHMANAN
Age/Gender : 44 Y 10 M 2 D /M
UHID/MR No : CVAL.0000050587
Visit ID : DKUMOPV7164
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 21/Aug/2026 12:09PM
Received : 21/Aug/2026 01:37PM
Reported : 21/Aug/2026 03:16PM
Status : Final Report
Client Name : PUP 24X7_CREDIT
Center location : 600056,Chennai

DEPARTMENT OF BIOCHEMISTRY

XPERT HEALTH ANNUAL HEALTH CHECK-UP

Test Name	Result	Unit	Bio. Ref. Interval	Method
HBA1C (GLYCATED HEMOGLOBIN) , WHOLE BLOOD EDTA				
HBA1C, GLYCATED HEMOGLOBIN	6.6	%		HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	143	mg/dL		Calculated

Comment:

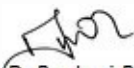
Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

Reference Group	HbA1c (%)	HbA1c (mmol/mol)
Non-Diabetic	<5.7	<38.8
Prediabetes	5.7 – 6.4	38.8 – 46.5
Diabetes	≥ 6.5	≥47.5
Diabetics		
Excellent control	6 – 7	42.1 – 53.0
Fair to good control	7 – 8	53.0 – 63.9
Unsatisfactory control	8 – 10	63.9 – 85.8
Poor control	>10	>85.8

Note: HbA1c IFCC (mmol/mol) = (10.93 x HbA1c NGSP (%)) – 23.50

- HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
- Trends in HbA1c values is a better indicator of Glycemic control than a single test.
- Low HbA1c in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
- Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
- In cases of Interference from Haemoglobin variants (HbF >25%, Homozygous Hemoglobinopathies) in HbA1C testing, alternative methods (Fructosamine) estimation is recommended for Glycemic Control. Abnormal Haemoglobin studies (HPLC/Electrophoresis) is recommended for detection of Hemoglobinopathies.

Page 2 of 7


Dr.Rashmi R
M.B.B.S.,M.D(Pathology)
Consultant Pathologist

SIN No:BI34148782



Patient Name : Mr.LAKSHMANAN
Age/Gender : 44 Y 10 M 2 D /M
UHID/MR No : CVAL.0000050587
Visit ID : DKUMOPV7164
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 21/Aug/2026 12:09PM
Received : 21/Aug/2026 01:37PM
Reported : 21/Aug/2026 03:06PM
Status : Final Report
Client Name : PUP 24X7_CREDIT
Center location : 600056,Chennai

DEPARTMENT OF BIOCHEMISTRY
XPERT HEALTH ANNUAL HEALTH CHECK-UP

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , SERUM				
TOTAL CHOLESTEROL	142	mg/dL	0-199	CHE/CHO/POD
TRIGLYCERIDES	48	mg/dL	60-145	Glycerol Phosphate Oxidase/peroxidase
HDL CHOLESTEROL	68	mg/dL	35-80	Selective Inhibition
NON-HDL CHOLESTEROL	74	mg/dL	<130	Calculated
LDL CHOLESTEROL	64.64	mg/dL	<100	Calculated (Friedewald)
VLDL CHOLESTEROL	9.56	mg/dL	<30	Calculated
CHOL / HDL RATIO	2.10		0-4.97	Calculated
ATHEROGENIC INDEX (AIP)	< 0.01		<0.11	Calculated

Comment:

Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.

Below Table as per Lipid Association of India (LAI) (2023) and Cardiological Society of India (CSI) (2024) Guidelines and Consensus Statements for Dyslipidemia Management:

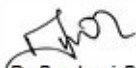
	Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
Total Cholesterol	< 200	< 200	200 – 239	≥ 240	≥ 240
Triglycerides	<150	< 150	150 – 199	200 – 499	≥ 500
LDL	< 100	100 – 129	130 – 159 Target for High Risk: < 70	160 – 189 Target for Very High Risk: < 50	≥ 190 Target for Extreme Risk – Category A,B :< 30, Category C: 10 – 15
HDL	≥ 60	≥ 60	M: <40, F: <50	<30	<30
Non-HDL Cholesterol	< 130	130 – 159	160-189 Target for High Risk: < 100	190 – 219 Target for Very High Risk: < 80	≥220 Target for Extreme Risk – Category A,B :< 60, Category C: 40 – 45

Note: Low risk – No known risk factor of cardiovascular disease. **Moderate risk** – Any one risk factor, eg; smoking/hypertension/diabetes mellitus etc. **High risk** – Two or more risk factors without any disease manifestation, chronic kidney disease, long-standing diabetes mellitus existing for >10 years, history of heterozygous familial hypercholesterolemia.

Very high risk – clinical evidence of coronary artery disease, long-standing diabetes mellitus existing for >20 years, etc. **Extremely high risk** – recurrent vascular events. (Category A- CAD with >1 feature of High risk group, Category B- CAD with >1 feature of very high risk group or poly-vascular disease, Category C- Recurrent CAD with other significant risk factors as high lipoprotein a, extensive coronary calcium)

- Measurements for Lipids (Especially Triglycerides) can show physiological (Dependent on diet, 10-12 hrs fasting pre-test condition) & analytical variations.
- Lipid Association of India (LAI) recommends screening of all adults (>20 yrs) for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors with lipid profile testing. The association recommends testing also to include Apolipoprotein B & Lipoprotein (a) for stratification and defining LDL – C targets.

Page 3 of 7


Dr. Rashmi R
M.B.B.S., M.D(Pathology)
Consultant Pathologist

SIN No:BI34148781



Patient Name : Mr.LAKSHMANAN
Age/Gender : 44 Y 10 M 2 D /M
UHID/MR No : CVAL.0000050587
Visit ID : DKUMOPV7164
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 21/Aug/2026 12:09PM
Received : 21/Aug/2026 01:37PM
Reported : 21/Aug/2026 03:06PM
Status : Final Report
Client Name : PUP 24X7_CREDIT
Center location : 600056,Chennai

DEPARTMENT OF BIOCHEMISTRY

XPERT HEALTH ANNUAL HEALTH CHECK-UP

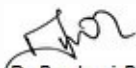
Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST (LFT) WITH GGT , SERUM				
BILIRUBIN, TOTAL	0.50	mg/dL	0-1.2	DIAZO METHOD
BILIRUBIN CONJUGATED (DIRECT)	0.10	mg/dL	0-0.4	Diazotized Sulfanilic
BILIRUBIN (INDIRECT)	0.40	mg/dL	0.0-1.1	Calculated
ALANINE AMINOTRANSFERASE (ALT/SGPT)	12.3	U/L	0-49	IFCC
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	14.5	U/L	0-46	IFCC
AST (SGOT) / ALT (SGPT) RATIO (DE RITIS)	1.2		<1.15	Calculated
ALKALINE PHOSPHATASE	32.80	U/L	40-129	
PROTEIN, TOTAL	7.00	g/dL	5.7-8.0	Biuret
ALBUMIN	4.20	g/dL	3.2-4.6	Bromocresol Green
GLOBULIN	2.80	g/dL	2.0-3.5	Calculated
A/G RATIO	1.5		0.9-2.0	Calculated
GAMMA GLUTAMYL TRANSPEPTIDASE (GGT)	9.90	U/L	10-45	Szasz

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin) Common patterns seen:

- Hepatocellular Injury: *AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries. *ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. Disproportionate increase in AST, ALT compared with ALP. AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1 In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilson's's diseases, Cirrhosis, but the increase is usually not >2. Note- If both SGPT and SGOT are within reference range then AST:ALT (De Ritis ratio) does not have any clinical significance.
- Cholestatic Pattern: *ALP – Disproportionate increase in ALP compared with AST, ALT. ALP elevation also seen in pregnancy, impacted by age and sex. *Bilirubin (Direct) and GGT elevated- helps to establish hepatic origin.
- Synthetic function impairment: *Albumin- Liver disease reduces albumin levels, Correlation with PT (Prothrombin Time) helps.
- Associated tests for assessment of liver fibrosis - Fibrosis-4 and APRI Index.

Page 4 of 7


Dr. Rashmi R
M.B.B.S., M.D (Pathology)
Consultant Pathologist

SIN No:BI34148781



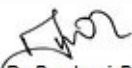
Patient Name : Mr.LAKSHMANAN
Age/Gender : 44 Y 10 M 2 D /M
UHID/MR No : CVAL.0000050587
Visit ID : DKUMOPV7164
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 21/Aug/2026 12:09PM
Received : 21/Aug/2026 01:37PM
Reported : 21/Aug/2026 03:06PM
Status : Final Report
Client Name : PUP 24X7_CREDIT
Center location : 600056,Chennai

DEPARTMENT OF BIOCHEMISTRY

XPERT HEALTH ANNUAL HEALTH CHECK-UP

Test Name	Result	Unit	Bio. Ref. Interval	Method
RENAL PROFILE/KIDNEY FUNCTION TEST (RFT/KFT) , SERUM				
CREATININE	0.70	mg/dL	0.62-1.17	Enzymatic
eGFR - ESTIMATED GLOMERULAR FILTRATION RATE	115.92	mL/min/1.73m ²	>60	CKD-EPI Formula 2021
UREA	19.90	mg/dL	10-50	Urease
BLOOD UREA NITROGEN	9.3	mg/dL	8.0 - 23.0	Calculated
URIC ACID	4.80	mg/dL	3.5-7.2	Uricase
CALCIUM	9.20	mg/dL	8.8-10.2	Arsenazo III
PHOSPHORUS, INORGANIC	4.10	mg/dL	2.5-4.5	Phosphomolybdate Complex
SODIUM	139	mmol/L	135-145	Direct ISE
POTASSIUM	4.2	mmol/L	3.5-5.1	ISE (Indirect)
CHLORIDE	100	mmol/L	98 - 107	Direct ISE
PROTEIN, TOTAL	7.00	g/dL	5.7-8.0	Biuret
ALBUMIN	4.20	g/dL	3.2-4.6	Bromocresol Green
GLOBULIN	2.80	g/dL	2.0-3.5	Calculated
A/G RATIO	1.5		0.9-2.0	Calculated


Dr.Rashmi R
M.B.B.S.,M.D(Pathology)
Consultant Pathologist

SIN No:BI34148781

Page 5 of 7



Patient Name : Mr.LAKSHMANAN
Age/Gender : 44 Y 10 M 2 D /M
UHID/MR No : CVAL.0000050587
Visit ID : DKUMOPV7164
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 21/Aug/2026 12:09PM
Received : 21/Aug/2026 04:02PM
Reported : 21/Aug/2026 07:05PM
Status : Final Report
Client Name : PUP 24X7_CREDIT
Center location : 600056,Chennai

DEPARTMENT OF IMMUNOLOGY

XPRT HEALTH ANNUAL HEALTH CHECK-UP

Test Name	Result	Unit	Bio. Ref. Interval	Method
THYROID PROFILE TOTAL (T3, T4, TSH) , SERUM				
TRI-iodothyronine (T3, TOTAL)	62.3	ng/dL	84.6-202	ECLIA
THYROXINE (T4, TOTAL)	4.89	µg/dL	5.12-14.06	ECLIA
TSH (Ultrasensitive/4thGen)	0.500	µIU/mL	0.270-4.20	ECLIA

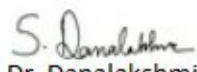
Comment:

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 - 3.0
Third trimester	0.3 - 3.0

1. TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH activates production of T3 (Triiodothyronine) & its prohormone T4 (Thyroxine). Increased blood level of T3 and T4 inhibit production of TSH.
2. TSH is elevated in primary hypothyroidism and will be low in primary hyperthyroidism. Elevated or low TSH in the context of normal free thyroxine is often referred to as sub-clinical hypo- or hyperthyroidism respectively.
3. Both T4 & T3 provides limited clinical information as both are highly bound to proteins in circulation and reflects mostly inactive hormone. Only a very small fraction of circulating hormone is free and biologically active. Isolated low T3 is often noticed in elderly
4. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, medication & circulating antibodies.

TSH	T3	T4	FT4	Possible Suggested Conditions correlating with pattern
High	Low	Low	Low	Primary Hypothyroidism, Post Thyroidectomy, Chronic Autoimmune Thyroiditis
High	N	N	N	Subclinical Hypothyroidism, Autoimmune Thyroiditis, Insufficient Hormone Treatment.
N/Low	Low	Low	Low	Secondary and Tertiary Hypothyroidism
Low	High	High	High	Primary Hyperthyroidism, Goitre, Thyroiditis, Drug effects, Early Pregnancy
Low	N	N	N	Subclinical Hyperthyroidism
Low	Low	Low	Low	Central Hypothyroidism, Treatment with Hyperthyroidism
Low	N	High	High	Thyroiditis, Interfering Antibodies
N/Low	High	N	N	T3 Thyrotoxicosis, Non thyroidal causes
High	High	High	High	Pituitary Adenoma; TSHoma/Thyrotropinoma

Page 6 of 7


Dr. Danalakshmi S.
M.D (Biochemistry)
Senior consultant Biochemistry



SIN No:IM12589212

This sample has been processed at Central Reference Laboratory , Sunny Side Services,8/17 Shafee Mohammed Road,Chennai-600006

Patient Name : Mr.LAKSHMANAN
Age/Gender : 44 Y 10 M 2 D /M
UHID/MR No : CVAL.0000050587
Visit ID : DKUMOPV7164
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 21/Aug/2026 12:09PM
Received : 21/Aug/2026 01:37PM
Reported : 21/Aug/2026 02:16PM
Status : Final Report
Client Name : PUP 24X7_CREDIT
Center location : 600056,Chennai

DEPARTMENT OF CLINICAL PATHOLOGY
XPERT HEALTH ANNUAL HEALTH CHECK-UP

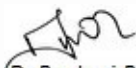
Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE URINE EXAMINATION (CUE) , URINE				
Physical Examination				
COLOUR	PALE YELLOW		PALE YELLOW	Visual
TRANSPARENCY	CLEAR		CLEAR	Physical Measurement
pH	5.50		5-7.5	Double Indicator
SP. GRAVITY	1.015		1.002-1.030	Bromothymol Blue
BIOCHEMICAL EXAMINATION				
URINE PROTEIN	NEGATIVE		NEGATIVE	Protein Error Of Indicator
GLUCOSE	NEGATIVE		NEGATIVE	Glucose Oxidase, Peroxidase
URINE BILIRUBIN	NEGATIVE		NEGATIVE	Diazonium salt
URINE KETONES (RANDOM)	NEGATIVE		NEGATIVE	Sodium Nitro Prusside
UROBILINOGEN	NORMAL		NORMAL	Diazonium salt
NITRITE	NEGATIVE		NEGATIVE	Diazotization
LEUCOCYTE ESTERASE	NEGATIVE		NEGATIVE	Leucocyte Esterase
CENTRIFUGED SEDIMENT WET MOUNT AND MICROSCOPY				
Pus Cells	2-3	/hpf	0-5	Microscopy
EPITHELIAL CELLS	1-2	/hpf	<10	Microscopy
RBC	NIL	/hpf	0-2	Microscopy
CASTS	ABSENT		0-2 Hyaline Cast	Microscopy
CRYSTALS	ABSENT		ABSENT	Microscopy

Comment:

All urine samples are checked for adequacy and suitability before examination. All abnormal chemical examination are rechecked and verified by manual methods. Microscopy findings are reported as an average of 10 high power fields.

*** End Of Report ***

Page 7 of 7


Dr.Rashmi R
M.B.B.S.,M.D(Pathology)
Consultant Pathologist

SIN No:C04871022

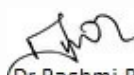


Patient Name : Mr.LAKSHMANAN
Age/Gender : 44 Y 10 M 2 D /M
UHID/MR No : CVAL.0000050587
Visit ID : DKUMOPV7164
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 21/Aug/2026 12:09PM
Received : 21/Aug/2026 01:37PM
Reported : 21/Aug/2026 02:16PM
Status : Final Report
Client Name : PUP 24X7_CREDIT
Center location : 600056,Chennai

TERMS AND CONDITIONS GOVERNING THIS REPORT

1. Reported results are for information and interpretation of the referring doctor or such other medical professionals, who understand reporting units, reference ranges and limitation of technologies. Laboratories not be responsible for any interpretation whatsoever
2. This is computer generated medical diagnostics report that has been validated by an Authorized Medical Practitioner/Doctor. The report does not need physical signature.
3. Partial reproduction of this report is not valid and should not be resorted to draw any conclusion.
4. In the case you are not the intended recipient of this report. Please immediately return the same to the concerned issuing desk. Any disclosure, copy or distribution of any contents of this report, is unlawful and is strictly prohibited.
5. Results delays may occur due to unforeseen circumstances such as non-availability of kits, equipment breakdown, natural calamities, IT downtime, logistic delays or any other unavoidable event. For certain tests based on analyte stability, criticality of results and in the interest of patient for having appropriate medical diagnosis, the same test may be outsourced to other accredited laboratory.
6. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen
7. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
8. The patient details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies
9. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only



Dr.Rashmi R
M.B.B.S.,M.D(Pathology)
Consultant Pathologist

SIN No:C04871022



Smart Health Report

Helping you understand
your health better

Curated for
Lakshmanan

Uhid
CVAL.0000050587

Curated Date
21-08-2026



Contents	Page no.
Overall Health Summary	<u>1</u> ↗
Your body systems	<u>2</u> ↗
Body systems in detail	
Blood Health	<u>3</u> ↗
Blood Glucose	<u>6</u> ↗
Heart Health	<u>9</u> ↗
Hormone Health	<u>13</u> ↗
Skin & Hair	<u>16</u> ↗
GI & Liver Health	<u>20</u> ↗
Kidney Health	<u>24</u> ↗
Parameters in detail	<u>28</u> ↗
Self-Care Tips	<u>42</u> ↗

Dear Lakshmanan,

You've just completed your Hemogram, Complete Urine Examination (Cue), Liver Function Test (Lft) With Ggt, Hba1c (Glycated Hemoglobin), Thyroid Profile Total (T3, T4, Tsh) etc tests, and we've updated your health summary with the latest insights. Regular check-ins like this help you stay informed and confident about your health—great job taking this step.

OVERALL HEALTH SUMMARY

Focusing on your blood, sugar, and thyroid health.

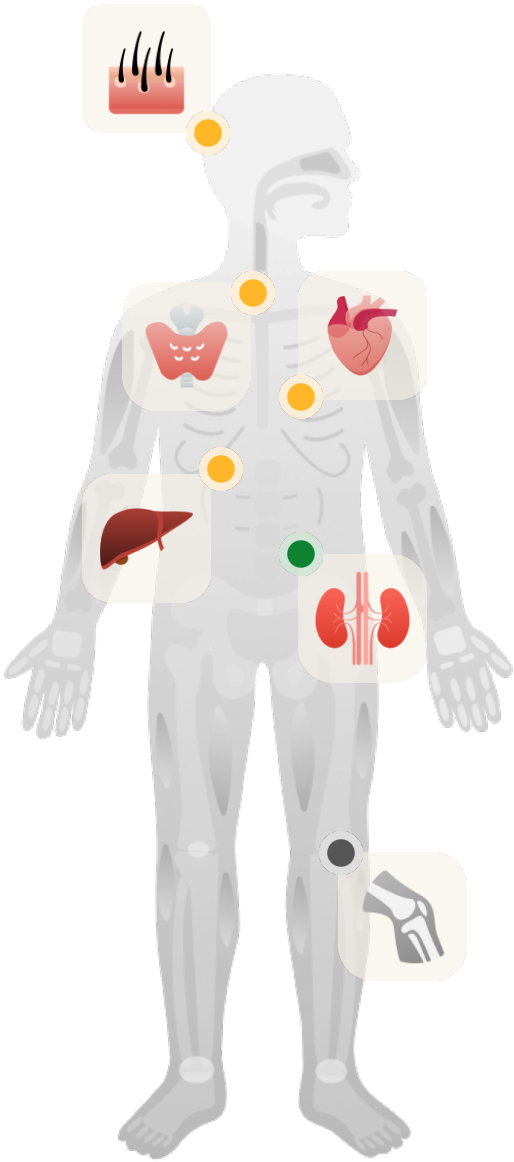
Your recent health check-up provides a snapshot of your well-being, highlighting areas for attention and those in good balance.

Here are some key insights from your results:

- Your blood count shows several parameters, including **Hemoglobin**, **Red Blood Cell Count**, **Packed Cell Volume (PCV)**, **MCH**, and **MCV**, are below their optimal ranges, while **RDW-CV** is elevated. This pattern suggests a form of anemia, possibly related to iron deficiency, which impacts your body's ability to carry oxygen.
- Your **Hba1C** is 6.6%, which indicates your average blood sugar levels over the past few months are elevated. This places you in the prediabetes range, a crucial time to consider lifestyle adjustments to support healthy glucose management.
- Your **Thyroxine (Total T4)** and **Tri-Iodothyronine (T3, Total)** levels are lower than expected, even though your **TSH** is within the normal range. This finding warrants further discussion with your doctor to understand your thyroid function more completely.

On a positive note, your kidney function, electrolyte balance, and liver enzyme levels are all within healthy ranges. Your lipid profile also looks favorable, with healthy cholesterol and triglyceride levels. Other parameters are being monitored for overall stability.

Your body systems — Simplified





Blood Health*

NEEDS ATTENTION

As on 21st Aug, '26



Blood Glucose*

NEEDS ATTENTION

As on 21st Aug, '26



Heart Health*

NEEDS MONITORING

As on 21st Aug, '26



Hormone Health*

NEEDS MONITORING

As on 21st Aug, '26



Skin & Hair

NEEDS MONITORING

As on 21st Aug, '26



GI & Liver Health*

NEEDS MONITORING

As on 21st Aug, '26



Kidney Health*

IDEAL HEALTH

As on 21st Aug, '26

OTHER BODY SYSTEMS



Bone & Muscle

NO DATA FOUND

Data for this organ isn't available yet



Vitamins

NO DATA FOUND

Data for this organ isn't available yet

* THIS INSIGHT IS BASED ON INCOMPLETE TEST DATA. SOME KEY PARAMETERS NEEDED TO FULLY ASSESS THIS ORGAN'S HEALTH WERE NOT AVAILABLE.



Blood Health

NEEDS ATTENTION As on 21st Aug, '26

Your RDW-CV needs a closer look.

Your blood health needs attention as your RDW-CV is out of range, and several other markers like Hemoglobin and MCH are borderline, indicating potential variations in red blood cell size and content.

- RDW-CV 16.4 %, Outside the normal range; may need medical attention.
- Hemoglobin 11.8 g/dL, Slightly outside the ideal range; worth monitoring.
- MCH (Mean Corpuscular HB) 26.9 pg, Slightly outside the ideal range; worth monitoring.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

- 1

Eat a balanced diet rich in iron and vitamins.
- 2

Include leafy greens and lean proteins in your meals.
- 3

Schedule your Complete Blood Count (CBC) / Hemogram by 2026-10-20 to recheck your RDW-CV levels.
- [View recommended tests](#)

↗

○

100%

Blood Health profile is complete



Blood Health

NEEDS ATTENTION As on 21st Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Blood Health

Blood Counts			● 1 OUT OF RANGE ● 6 BORDERLINE ✓ 12 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
RDW-CV	● 16.4 % on 21st Aug, '26	N/A	
Hemoglobin	● 11.8 g/dL on 21st Aug, '26	N/A	
MCH (Mean Corpuscular HB)	● 26.9 pg on 21st Aug, '26	N/A	
Packed Cell Volume (PCV)	● 36.30 % on 21st Aug, '26	N/A	
Red Blood Cell Count	● 4.4 Million/cu.mm on 21st Aug, '26	N/A	
Total Leucocytes Count (WBC)	● 5,500 cells/cu.mm on 21st Aug, '26	N/A	
Mean Cell Volume (MCV)	● 82.4 fL on 21st Aug, '26	N/A	
Absolute Basophil Count (ABC)	✓ 5.5 Cells/cu.mm on 21st Aug, '26	N/A	
Basophils	✓ 0.1 % on 21st Aug, '26	N/A	
Absolute Monocyte Count (AMC)	✓ 242 Cells/cu.mm on 21st Aug, '26	N/A	
Eosinophils	✓ 1.4 % on 21st Aug, '26	N/A	
Absolute Lymphocyte Count (ALC)	✓ 2046 Cells/cu.mm on 21st Aug, '26	N/A	
Platelet Count	✓ 219000 cells/cu.mm on 21st Aug, '26	N/A	
Absolute Neutrophil Count (ANC)	✓ 3129.5 Cells/cu.mm on 21st Aug, '26	N/A	
MCHC (Mean Corpuscular HB Concentration)	✓ 32.6 g/dL on 21st Aug, '26	N/A	

Lymphocytes	✓ 37.2 % on 21st Aug, '26	N/A
Monocytes	✓ 4.4 % on 21st Aug, '26	N/A
Neutrophils	✓ 56.9 % on 21st Aug, '26	N/A
Absolute Eosinophil Count (AEC)	✓ 77 Cells/cu.mm on 21st Aug, '26	N/A



Blood Glucose

NEEDS ATTENTION As on 21st Aug, '26

Your HbA1c is outside the normal range.

Your blood glucose needs attention because your HbA1c is out of range, indicating suboptimal sugar control. Your Estimated Average Glucose is also borderline.

- Hba1C or Glycated Hemoglobin 6.6 %, Outside the normal range; may need medical attention.
- Estimated Average Glucose (EAG) 143 mg/dL, Slightly outside the ideal range; worth monitoring.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

- 1 Engage in regular physical activity, such as walking or cycling.
- 2 Choose whole grains and limit sugary drinks.
- 3 Schedule your Diabetes Profile / Blood Sugar Test by 2027-02-17 to recheck your Hba1C or Glycated Hemoglobin levels.

[View recommended tests](#) ↗

50%

Blood Glucose assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Blood Glucose

NEEDS ATTENTION As on 21st Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Blood Glucose

Sugar 1 OUT OF RANGE 1 BORDERLINE		
PARAMETER NAME	RECENT VALUE	PAST VALUE
Hba1C or Glycated Hemoglobin	6.6 % on 21st Aug, '26	N/A
Estimated Average Glucose (EAG)	143 mg/dL on 21st Aug, '26	N/A

 50%

Your Blood Glucose assessment is based on partial data

Here are the key parameters needed to complete your Blood Glucose health profile:

1

These essential parameters directly measure your Blood Glucose's health. We recommend testing them to get a clear and complete picture of its current function

- Glucose, Post Prandial (Pp)
- Glucose, Fasting



Ready to complete your health picture?

Add all recommended tests to your cart to get an up-to-date health analysis

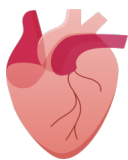
[Book tests now](#) ↗



Have recent reports from other labs?

Upload your existing reports to complete your health picture without needing new tests

[Upload now](#) ↗



Heart Health

NEEDS MONITORING As on 21st Aug, '26

Your Triglycerides are slightly elevated.

Your heart health is borderline due to your Triglycerides being slightly outside the ideal range.

- Triglycerides - Serum 48 mg/dL, Slightly outside the ideal range; worth monitoring.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

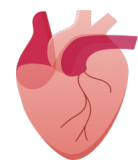
- 1 Aim for at least 30 minutes of moderate exercise most days.
- 2 Reduce intake of saturated and trans fats.
- 3 Schedule your Lipid Profile by 2027-05-18 to recheck your Triglycerides - Serum levels.

[View recommended tests](#) ↗

79%

Heart Health assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Heart Health

NEEDS MONITORING As on 21st Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Heart Health

Lipids			1 BORDERLINE	6 IN RANGE
PARAMETER NAME	RECENT VALUE		PAST VALUE	
Triglycerides - Serum	48 mg/dL	on 21st Aug, '26	N/A	
Total Cholesterol - Serum	142 mg/dL	on 21st Aug, '26	N/A	
LDL Cholesterol - Serum	64.64 mg/dL	on 21st Aug, '26	N/A	
HDL Cholesterol - Serum	68 mg/dL	on 21st Aug, '26	N/A	
Non-HDL Cholesterol	74 mg/dL	on 21st Aug, '26	N/A	
Total Cholesterol / HDL Ratio	2.10	on 21st Aug, '26	N/A	
VLDL Cholesterol - Serum	9.56 mg/dL	on 21st Aug, '26	N/A	

INDIRECT TESTS FOR AFFECTING YOUR HEART HEALTH

Here are a few tests that might be risk markers for your Heart Health

1 Hemoglobin

Too high thickens blood, raising blood pressure and clot risk. Too low reduces oxygen supply, weakening the heart.

As on 21st Aug, 2026, this value was 11.8 g/dL out of range

2 Estimated Average Glucose (EAG)

High EAG damages blood vessels, increasing heart disease and stroke risk.

As on 21st Aug, 2026, this value was 143 mg/dL out of range

3 Hba1C or Glycated Hemoglobin

High HbA1c indicates prolonged high blood sugar, leading to artery damage and higher heart attack risk.

As on 21st Aug, 2026, this value was 6.6 % out of range

 79%

Your Heart assessment is based on partial data

Here are the key parameters needed to complete your Heart health profile:

1

These essential parameters directly measure your Heart's health. We recommend testing them to get a clear and complete picture of its current function

- Apolipoprotein B

2

These critical parameters help us evaluate key risks to your Heart from other body systems. We recommend testing them for a complete risk profile.

- Glucose, Fasting



Ready to complete your health picture?

Add all recommended tests to your cart to get an up-to-date health analysis

[Book tests now](#) ↗



Have recent reports from other labs?

Upload your existing reports to complete your health picture without needing new tests

[Upload now](#) ↗



Hormone Health

NEEDS MONITORING As on 21st Aug, '26

Your T3 levels need attention.

Your hormone health is borderline because your Tri-Iodothyronine (T3) is out of range and Thyroxine (T4) is borderline, which may indicate reduced thyroid activity.

- Tri-Iodothyronine (T3, Total), Serum 62.3 ng/dL, Outside the normal range; may need medical attention.
- Thyroxine (Total T4), Serum 4.89 µg/dL, Slightly outside the ideal range; worth monitoring.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

1 Practice stress-reducing activities like meditation or yoga.

2 Ensure adequate intake of iodine and selenium.

3 Schedule your Thyroid Profile by 2027-02-17 to recheck your Tri-Iodothyronine (T3, Total), Serum levels.

[View recommended tests](#) ↗

10%

Hormone Health assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Hormone Health

NEEDS MONITORING As on 21st Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Hormone Health

Thyroid			● 1 OUT OF RANGE ● 1 BORDERLINE ✓ 1 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Tri-Iodothyronine (T3, Total), Serum	● 62.3 ng/dL on 21st Aug, '26	N/A	
Thyroxine (Total T4), Serum	● 4.89 µg/dL on 21st Aug, '26	N/A	
Thyroid Stimulating Hormone (TSH), Serum	✓ 0.500 µIU/mL on 21st Aug, '26	N/A	

10% Your Hormone assessment is based on partial data

Here are the key parameters needed to complete your Hormone health profile:

1

These essential parameters directly measure your Hormone's health. We recommend testing them to get a clear and complete picture of its current function

- Testosterone serum
- Follicular Stimulating Hormone (FSH)
- Free T4 (FT4)
- Free T3 (FT3)
- Prolactin
- Leutinising Hormone (LH)
- Anti Thyroglobulin Antibody
- Parathyroid Hormone [PTH]

2

These critical parameters help us evaluate key risks to your Hormone from other body systems. We recommend testing them for a complete risk profile.

- Calcium, Serum



Ready to complete your health picture?

Add all recommended tests to your cart to get an up-to-date health analysis

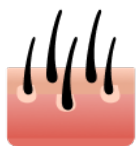
[Book tests now](#) ↗



Have recent reports from other labs?

Upload your existing reports to complete your health picture without needing new tests

[Upload now](#) ↗



Skin & Hair

NEEDS MONITORING As on 21st Aug, '26

Your WBC count is slightly outside range.

Your skin and hair health is borderline due to your Total Leucocytes Count (WBC) being borderline.

- Total Leucocytes Count (WBC) 5,500 cells/cu.mm, Slightly outside the ideal range; worth monitoring.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

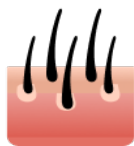
- 1 Stay well-hydrated by drinking plenty of water throughout the day.
- 2 Consume a balanced diet rich in vitamins and minerals.
- 3 Schedule your Complete Blood Count (CBC) / Hemogram by 2026-10-04 to recheck your Total Leucocytes Count (WBC) levels.

[View recommended tests](#) ↗

C 58%

Skin & Hair assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Skin & Hair

NEEDS MONITORING As on 21st Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Skin & Hair

Blood Counts			● 1 BORDERLINE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Total Leucocytes Count (WBC)	● 5,500 cells/cu.mm on 21st Aug, '26	N/A	

INDIRECT TESTS FOR AFFECTING YOUR SKIN & HAIR

Here are a few tests that might be risk markers for your Skin & Hair

1 Hemoglobin

Low hemoglobin (anemia) reduces oxygen supply to hair follicles and skin, causing hair thinning, dull skin, and increased hair loss.

*As on 21st Aug, 2026, **this value was 11.8 g/dL out of range***

C 58%

Your Skin and Hair assessment is based on partial data

Here are the key parameters needed to complete your Skin and Hair health profile:

1

These essential parameters directly measure your Skin and Hair's health. We recommend testing them to get a clear and complete picture of its current function

- Testosterone serum

2

These critical parameters help us evaluate key risks to your Skin and Hair from other body systems. We recommend testing them for a complete risk profile.

- Vitamin B12 -Serum



Ready to complete your health picture?

Add all recommended tests to your cart to get an up-to-date health analysis

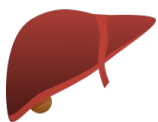
[Book tests now](#) ↗



Have recent reports from other labs?

Upload your existing reports to complete your health picture without needing new tests

[Upload now](#) ↗



GI & Liver Health

NEEDS MONITORING As on 21st Aug, '26

Your liver enzymes are slightly elevated.

Your GI and Liver health is borderline due to Alkaline Phosphatase (ALP) and Gamma Glutamyl Transferase (GGT) being borderline, which may indicate mild strain on liver processing.

- Alkaline Phosphatase (ALP) 32.80 U/L, Slightly outside the ideal range; worth monitoring.
- Gamma Glutamyl Transferase (GGT) 9.90 U/L, Slightly outside the ideal range; worth monitoring.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

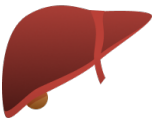
- 1 Limit alcohol consumption to support liver health.
- 2 Focus on a diet rich in fruits, vegetables, and lean proteins.
- 3 Schedule your Liver Function Test (LFT) by 2027-02-17 to recheck your Alkaline Phosphatase (ALP) levels.

[View recommended tests](#) ↗

C 68%

GI & Liver Health assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



GI & Liver Health

NEEDS MONITORING As on 21st Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding GI & Liver Health

Liver Function Test (LFT)			● 2 BORDERLINE ✓ 9 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Alkaline Phosphatase (ALP)	● 32.80 U/L on 21st Aug, '26	N/A	
Gamma Glutamyl Transferase (GGT)	● 9.90 U/L on 21st Aug, '26	N/A	
Bilirubin Direct	✓ 0.10 mg/dL on 21st Aug, '26	N/A	
Bilirubin-Indirect	✓ 0.40 mg/dL on 21st Aug, '26	N/A	
Bilirubin Total	✓ 0.50 mg/dL on 21st Aug, '26	N/A	
Alanine Transaminase (SGPT / ALT)	✓ 12.3 U/L on 21st Aug, '26	N/A	
Aspartate Aminotransferase (AST / SGOT)	✓ 14.5 U/L on 21st Aug, '26	N/A	
Albumin/Globulin Ratio	✓ 1.5 on 21st Aug, '26	1.5 on 21st Aug, '26	
Globulin	✓ 2.80 g/dL on 21st Aug, '26	N/A	
Albumin	✓ 4.20 g/dL on 21st Aug, '26	N/A	
Proteins	✓ 7.00 g/dL on 21st Aug, '26	N/A	

INDIRECT TESTS FOR AFFECTING YOUR GI & LIVER HEALTH

Here are a few tests that might be risk markers for your GI & Liver Health

1 Hemoglobin

Low levels reduce oxygen to the gut, causing weak digestion and infection risk. High levels may affect blood flow, leading to digestive issues.

*As on 21st Aug, 2026, **this value was 11.8 g/dL out of range***

C 68%

Your GI and Liver assessment is based on partial data

Here are the key parameters needed to complete your GI and Liver health profile:

1

These essential parameters directly measure your GI and Liver's health. We recommend testing them to get a clear and complete picture of its current function

- Urobilinogen
- Amylase
- Lipase
- Bile Pigment
- Urine Bilirubin



Ready to complete your health picture?

Add all recommended tests to your cart to get an up-to-date health analysis

[Book tests now](#) ↗



Have recent reports from other labs?

Upload your existing reports to complete your health picture without needing new tests

[Upload now](#) ↗



Kidney Health

IDEAL HEALTH As on 21st Aug, '26

Your kidney function is looking great!

Your kidney health is ideal, with all key parameters within the healthy range.

- Creatinine 0.70 mg/dL, Within the healthy range.
- Estimated glomerular filtration rate(eGFR) 115.92 mL/min/1.73m², Within the healthy range.
- Sodium - Serum 139 mmol/L, Within the healthy range.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

1 Continue to stay well-hydrated throughout the day.

2 Maintain a balanced diet with controlled sodium intake.

C 45%

Kidney Health assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Kidney Health

IDEAL HEALTH As on 21st Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Kidney Health

Kidney Function Test (KFT)

✓ 4 IN RANGE

PARAMETER NAME	RECENT VALUE	PAST VALUE
Creatinine	✓ 0.70 mg/dL on 21st Aug, '26	N/A
Estimated glomerular filtration rate(eGFR)	✓ 115.92 mL/min/1.73m² on 21st Aug, '26	N/A
Urea	✓ 19.90 mg/dL on 21st Aug, '26	N/A
Blood Urea Nitrogen	✓ 9.3 mg/dL on 21st Aug, '26	N/A

Electrolytes

✓ 3 IN RANGE

PARAMETER NAME	RECENT VALUE	PAST VALUE
Sodium - Serum	✓ 139 mmol/L on 21st Aug, '26	N/A
Potassium - Serum	✓ 4.2 mmol/L on 21st Aug, '26	N/A
Chloride - Serum	✓ 100 mmol/L on 21st Aug, '26	N/A

Minerals

✓ 1 IN RANGE

PARAMETER NAME	RECENT VALUE	PAST VALUE
Phosphorus, Inorganic - Serum	✓ 4.10 mg/dL on 21st Aug, '26	N/A

INDIRECT TESTS FOR AFFECTING YOUR KIDNEY HEALTH

Here are a few tests that might be risk markers for your Kidney Health

1 Estimated Average Glucose (EAG)

High EAG suggests prolonged high blood sugar, which can damage kidney blood vessels and lead to kidney disease.

As on 21st Aug, 2026, this value was 143 mg/dL out of range

2 Hba1C or Glycated Hemoglobin

High HbA1c indicates chronic high blood sugar, which damages kidney filtration ability.

As on 21st Aug, 2026, this value was 6.6 % out of range

 45%

Your Kidney assessment is based on partial data

Here are the key parameters needed to complete your Kidney health profile:

1

These essential parameters directly measure your Kidney's health. We recommend testing them to get a clear and complete picture of its current function

- Calcium, Serum
- Ketones
- Crystals
- Bacteria
- Parasites
- Leukocyte Esterase
- Urine Blood
- Casts
- Epithelial Cells
- Urine Protein

2

These critical parameters help us evaluate key risks to your Kidney from other body systems. We recommend testing them for a complete risk profile.

- Red Blood Cells
- Glucose (Urine Sugar)



Ready to complete your health picture?

Add all recommended tests to your cart to get an up-to-date health analysis

[Book tests now](#) ↗



Have recent reports from other labs?

Upload your existing reports to complete your health picture without needing new tests

[Upload now](#) ↗

Let's understand the
critical markers in
detail that require
your focus

 **T3**
21st Aug, 2026

62.3 ng/dL
OUT OF RANGE

- T3 is an active thyroid hormone. This test evaluates thyroid function and can help diagnose thyroid disorders.
- High T3 means more thyroid hormone is circulating than usual. This may cause restlessness, heat intolerance, or fast heartbeat. If TSH is low, it adds context. Review helps guide balance.

Reference range

Your result (62.3 ng/dL) is out of range low

Inference	Value (ng/dL)
Out of range Low	<75
Borderline Low	75-80
Ideal (Normal Range)	80-200



RBC Count

21st Aug, 2026

4.4

Million/cu.mm

BORDERLINE

- This test counts how many red blood cells you have, which carry oxygen to your organs and muscles.
- A slightly low RBC count means you have a few fewer red cells than usual. This may happen after low iron foods, recent illness, or extra fluids. If hemoglobin or PCV are also low, that supports it. Usually not urgent, but repeating helps track.

**WBC**

21st Aug, 2026

5,500
cells/cu.mm
BORDERLINE

- This test counts white blood cells, which defend your body against infections and illnesses.
- A slightly low WBC may follow viral illness, stress, or medicines. If neutrophils or lymphocytes are also low, that supports it. Usually not urgent, but worth repeating.



T4

21st Aug, 2026

4.89 $\mu\text{g/dL}$

BORDERLINE

- T4 is a thyroid hormone that regulates metabolism. This test evaluates thyroid function and can diagnose hypothyroidism or hyperthyroidism.
- Slightly low T4 means there is a bit less thyroid hormone circulating. This may follow illness or under-replacement. If TSH is raised, that supports it. Changes show slowly, so repeating is helpful.

Reference range

Your result (4.89 $\mu\text{g/dL}$) is **borderline low**

Inference	Value ($\mu\text{g/dL}$)
Out of range Low	<4.5
Borderline Low	4.5-5
Ideal (Normal Range)	5-11.5



eAG

21st Aug, 2026

143 mg/dL

BORDERLINE

- EAG translates HbA1c results into an estimated daily blood sugar average over 2-3 months. It helps track long-term glucose control in diabetes management.
- High EAG suggests prolonged high blood sugar, which can damage kidney blood vessels and lead to kidney disease.

Reference range

Your result (143 mg/dL) is **borderline high**

Inference	Value (mg/dL)
Out of range High	>154
Borderline High	126-154
Ideal (Normal Range)	70-126



ALP

21st Aug, 2026

32.80 U/L

BORDERLINE

- The Alkaline Phosphatase (ALP) test measures ALP enzyme in the blood, which plays a role in bone formation and liver function. This test helps detect liver disease, bone disorders, and bile duct obstruction early.
- A slightly low ALP may follow low protein, low zinc, or fasting. If albumin is also low, that supports it. Usually not urgent, but worth repeating if it stays low.

**GGT**

21st Aug, 2026

9.90 U/L**BORDERLINE**

- GGT is an enzyme in the liver that helps break down toxins. This test helps detect liver disease, bile duct issues, and alcohol-related liver damage.
- A low GGT is usually not concerning and often normal. No action is needed if other liver tests are fine.



Hba1C

21st Aug, 2026

6.6 %
OUT OF RANGE

- HbA1C measures the percentage of sugar attached to hemoglobin in red blood cells. It reflects average blood sugar levels over 2-3 months and helps diagnose and monitor diabetes.
- High HbA1c indicates chronic high blood sugar, which damages kidney filtration ability.

Reference range

Your result (6.6 %) is out of range high

Inference	Value (%)
Out of range High	>6.4
Borderline High	5.7-6.4
Ideal (Normal Range)	4-5.7



MCH

21st Aug, 2026

26.9 pg

BORDERLINE

- This test checks how much hemoglobin your red blood cells contain on average.
- A slightly low MCH means each red cell carries a bit less hemoglobin. This may follow low iron foods or recent illness. If MCV or ferritin are also low, that supports it. Usually not urgent, but worth repeating.

Reference range

Your result (26.9 pg) is **borderline low**

Inference	Value (pg)
Out of range Low	<26
Borderline Low	26-27
Ideal (Normal Range)	27-31



Hemoglobin

21st Aug, 2026

11.8 g/dL
BORDERLINE

- The Hemoglobin test measures the oxygen-carrying protein in red blood cells, essential for oxygen transport.
- Low levels reduce oxygen to the gut, causing weak digestion and infection risk. High levels may affect blood flow, leading to digestive issues.

Reference range

Your result (11.8 g/dL) is **borderline low**

Inference	Value (g/dL)
Critical Low	<=6
Out of range Low	6-11
Borderline Low	11-12
Ideal (Normal Range)	12-16.5



RDW-CV

21st Aug, 2026

16.4 %

OUT OF RANGE

- This test calculates the percentage variation in red blood cell size, helping detect nutrient deficiencies or blood disorders.
- Very high RDW-CV means red cells vary a lot in size. You may feel weak, lightheaded, or short of breath. If Hb or MCV are also abnormal, this suggests anemia and should be checked.

Reference range

Your result (16.4 %) is critical high

Inference	Value (%)
Critical High	>=16.1
Out of range High	16-16.1
Borderline High	14.5-16
Ideal (Normal Range)	11.5-14.5



MCV

21st Aug, 2026

82.4 fL

BORDERLINE

- This test measures the average size of your red blood cells.
- A slightly low MCV means red cells are a bit smaller. This may follow iron shortage or poor diet. If Hb or ferritin are also low, that supports it. Worth repeating, as it often improves with time.



Triglycerides

21st Aug, 2026

48 mg/dL

BORDERLINE

- This test measures the levels of triglycerides, a type of fat in the blood. High levels may increase the risk of heart disease.
- A slightly low triglyceride level may follow fasting, light diet, or medicines. If HDL is normal, this is not a concern. Usually not urgent.



PCV

21st Aug, 2026

36.30 %

BORDERLINE

- The PCV test shows what percentage of your blood is made up of red blood cells, which carry oxygen.
- A slightly low PCV means the proportion of red cells is a bit reduced. This may follow low iron foods, recent illness, or fluid overload. If Hb or MCV are also low, that supports it. Worth repeating to track.



General recommendations

- Exercise regularly; do strength exercises (e.g. weight lifting) and stretching exercises (e.g. yoga) 2-3 times a week.
- Try getting at least 150 minutes of exercise each week and don't go over 2 days in a row without some form of physical activity.
- Opt for a healthy meal plan consisting of fresh fruits and vegetables, lean protein, low-fat dairy products, complex carbohydrates, and healthy fats.
- Manage stress and maintain a healthy weight.
- Check your feet and skin regularly for redness, cuts, bruises, blisters and sores.

YOUR CLINICAL PROFILE

We currently don't have any information about your health conditions, medications, or surgical history.

Help Us Understand Your Health Better

Complete your health profile by adding details such as health conditions, medicines, allergies, past surgeries, etc., to unlock deeper, more accurate, and personalized insights in your Smart Report.

[Complete Your Health Profile >](#)